Drug Coverage Policy

Effective Date.....9/15/2025
Coverage Policy Number IP0582
Policy Title.....Inpefa

Inpefa

- Inpefa™ (sotagliflozin tablets - Lexicon)

INSTRUCTIONS FOR USE
The following Coverage Policy applies to health benefit plans administered by Cigna Companies. Certain Cigna Companies and/or lines of business only provide utilization review services to clients and do not make coverage determinations. References to standard benefit plan language and coverage determinations do not apply to those clients. Coverage Policies are intended to provide guidance in interpreting certain standard benefit plans administered by Cigna Companies. Please note, the terms of a customer's particular benefit plan document [Group Service Agreement, Evidence of Coverage, Certificate of Coverage, Summary Plan Description (SPD) or similar plan document] may differ significantly from the standard benefit plans upon which these Coverage Policies are based. For example, a customer's benefit plan document may contain a specific exclusion related to a topic addressed in a Coverage Policy. In the event of a conflict, a customer's benefit plan document always supersedes the information in the Coverage Policies. In the absence of a controlling federal or state coverage mandate, benefits are ultimately determined by the terms of the applicable benefit plan document. Coverage determinations in each specific instance require consideration of 1) the terms of the applicable benefit plan document in effect on the date of service; 2) any applicable laws/regulations; 3) any relevant collateral source materials including Coverage Policies and; 4) the specific facts of the particular situation. Each coverage request should be reviewed on its own merits. Medical directors are expected to exercise clinical judgment where appropriate and have discretion in making individual coverage determinations. Where coverage for care or services does not depend on specific circumstances, reimbursement will only be provided if a requested service(s) is submitted in accordance with the relevant criteria outlined in the applicable Coverage Policy, including covered diagnosis and/or procedure code(s). Reimbursement is not allowed for services when billed for conditions or diagnoses that are not covered under this Coverage Policy (see "Coding Information" below). When billing, providers must use the most appropriate codes as of the effective date of the submission. Claims submitted for services that are not accompanied by covered code(s) under the applicable Coverage Policy will be denied as not covered. Coverage Policies relate exclusively to the administration of health benefit plans. Coverage Policies are not recommendations for treatment and should never be used as treatment guidelines. In certain markets, delegated vendor guidelines may be used to support medical necessity and other coverage determinations.

OVERVIEW

Inpefa, a sodium glucose co-transporter-2 (SGLT-2) and SGLT-1 inhibitor, is indicated **to reduce the risk of cardiovascular (CV) death, hospitalization for heart failure (HHF), and urgent heart failure visit in adults** with[1]:

- Heart failure; OR
- Type 2 diabetes mellitus, chronic kidney disease (CKD), and other CV risk factors.

Unlike other SGLT-2 inhibitors, Inpefa is not indicated for glycemic control.

Guidelines

SGLT-2 inhibitors are recommended as first-line treatment for heart failure in the American Heart Association/American College of Cardiology (ACC)/Heart Failure Society of America Guideline for the Management of Heart Failure (2022).[2] The pivotal trial with Inpefa in patients with heart failure is noted to extend the benefits of SGLT-2 inhibitors to patients with diabetes and acutely decompensated heart failure.[5] Inpefa before or shortly after discharge reduced CV mortality and hospitalization in SOLOIST-WHF.

The ACC Expert Consensus Decision Pathway on Management of HFpEF (2023) notes the benefit of SGLT-2 inhibitors as part of guideline-directed medical therapy in patients with heart failure with preserved ejection fraction (HFpEF).[8] According to the statement, SGLT-2 inhibitors (dapagliflozin, Jardiance™ [empagliflozin tablets]) should be initiated in all individuals with HFpEF who are stable during hospitalization and have no contraindications. The pivotal heart failure trial with Inpefa is mentioned, and it is noted that Inpefa treatment resulted in a significantly lower total number of deaths from CV causes, HHF, and urgent visits for heart failure than placebo, regardless of left ventricular ejection fraction.

A 2023 Focused Update of the 2021 European Society of Cardiology Guidelines for the Diagnosis and Treatment of Acute and Chronic Heart Failure recommends an SGLT-2 inhibitor (dapagliflozin, Jardiance) in patients with symptomatic heart failure with moderately-reduced ejection fraction (HFmrEF) or HFpEF to reduce the risk of HHF or CV death (Class I, Level A).[10] In patients hospitalized for acute heart failure, an intensive strategy of initiation and rapid up-titration of evidence-based treatment (including, but not limited to dapagliflozin or Jardiance) before discharge and during frequent and careful follow-up visits in the first 6 weeks following HHF is recommended to reduce the risk of HHF or death (Class I, Level B). In patients with type 2 diabetes and CKD, SGLT-2 inhibitors (dapagliflozin or Jardiance) are recommended to reduce the risk of HHF or CV death (Class I, Level A).

The 2024 ACC Expert Consensus Decision Pathway for Heart Failure with Reduced Ejection Fraction (HFrEF) recommends SGLT inhibitors (dapagliflozin, Inpefa, and Jardiance) as a core therapy in the "four pillars" of medical care for HFrEF.[11]

The Kidney Diseases: Improving Global Outcomes (KDIGO) clinical practice guideline for diabetes management in CKD (2022) and the American Diabetes Association (ADA)/KDIGO diabetes management in CKD consensus report (2022) recommend an SGLT-2 with proven kidney or CV benefit for patients with type 2 diabetes, CKD, and estimated glomerular filtration rate (eGFR) ≥ 20 mL/min/1.73 m^2.[4,6] SGLT-2 inhibitors are recommended independent of hemoglobin A_{1c} (HbA_{1c}) or the need for additional glucose lowering. This recommendation is based on strong evidence that SGLT-2 inhibitors reduce CKD progression, heart failure, and atherosclerotic CV disease (ASCVD) risk in patients with type 2 diabetes and CKD. Results from pivotal trials with Inpefa are briefly mentioned. The KDIGO Clinical Practice Guideline for the Evaluation and Management of Chronic Kidney Disease (2024) reaffirms the statements in the KDIGO 2022 Clinical Practice Guidelines for Diabetes Management in CKD.[12] In addition, more general recommendations for adults with CKD (without diabetes) are made.

The ADA standards of care (2025) recommend an SGLT-2 inhibitor or glucagon-like peptide-1 (GLP-1) agonist with demonstrated CV disease benefit as part of the glucose lowering regimen and comprehensive CV risk reduction strategy, independent of HbA_{1c} in patients with established CV disease or indicators of high CV risk, established kidney disease, or heart failure.[3] In people with type 2 diabetes and established heart failure with either preserved or reduced ejection fraction, an SGLT2 inhibitor (including Inpefa) with proven benefit in this population is recommended to reduce the risk of worsening heart failure and cardiovascular death. The data with Inpefa from its pivotal trials (SCORED and SOLOIST-WHF) are mentioned and it is noted that both trials were terminated early due to a loss of funding. In SOLOIST-WHF, when initiated during or shortly after hospitalization in people with diabetes, Inpefa reduced the risk of the primary endpoint of deaths from CV causes and hospitalizations and urgent visits for heart failure. Early termination of SOLOIST-WHF limited the ability to determine the effects of Inpefa in HFpEF specifically. In SCORED, the primary endpoint of deaths from CV causes, HHF, and urgent visits for heart failure were reduced in patients with type 2 diabetes, CKD, and risks for CV disease treated with Inpefa.

The American Association of Clinical Endocrinology (AACE) comprehensive type 2 diabetes management algorithm (2023) builds on the 2022 AACE diabetes clinical practice guideline.[5,7] Inpefa is not addressed. However, SGLT-2 inhibitors with 'proven benefit' are an alternative to GLP-1 agonists to reduce the risk of major adverse CV events or CV death in patients with type 2 diabetes and established CV disease. For patients with type 2 diabetes and established ASCVD or at high risk for ASCVD, the use of an SGLT-2 inhibitor reduces the risk of HHF and in patients with heart failure and/or CKD, SGLT-2 inhibitors should be used as first-line therapy. SGLT-2 inhibitors are recommended in patients with type 2 diabetes and heart failure regardless of glycemic goal or other antihyperglycemic treatments. There are robust data for the benefit of SGLT-2 inhibitors to reduce adverse renal outcomes. Use of an SGLT-2 inhibitor with 'proven benefits' is recommended as initial therapy to reduce the progression of diabetic kidney disease and CV disease risk for patients with type 2 diabetes and diabetic kidney disease with eGFR ≥ 25 mL/min/1.73 m^2 or ≥ 20 mL/min/1.73 m^2 if heart failure is also present.

Coverage Policy

POLICY STATEMENT

Prior Authorization is required for benefit coverage of Inpefa. All approvals are provided for the duration noted below.

Inpefa is considered medically necessary when the following criteria are met:

FDA-Approved Indication(s)

1. **Heart Failure, To Reduce the Risk of Cardiovascular Death, Hospitalization for Heart Failure, and Urgent Heart Failure Visit.** Approve for 1 year if the patient is ≥ 18 years of age and preferred product criteria is met for the product(s) as listed in the below table(s).

2. **Type 2 Diabetes, To Reduce The Risk of Cardiovascular Death, Hospitalization for Heart Failure, and Urgent Heart Failure Visit.** Approve for 1 year if the patient meets ALL of the following (A, B, C, <u>and</u> D):
   A) Patient is ≥ 18 years of age; AND
   B) Patient has chronic kidney disease; AND
   C) Patient has one or more cardiovascular risk factor(s), according to the prescriber.
   <u>Note</u>: Patients with heart failure should be reviewed under criteria for *Heart Failure.*
   D) Preferred product criteria are met for the product(s) as listed in the below table(s)

Employer Plans:

Product	Criteria
Inpefa (sotagliflozin)	Patient has tried, and according to the prescriber, experienced inadequate efficacy, OR significant intolerance with **BOTH** Farxiga and Jardiance.

Individual and Family Plans:

Product	Criteria
Inpefa (sotagliflozin)	Patient has tried, and according to the prescriber, experienced inadequate efficacy, OR significant intolerance with **BOTH** Farxiga and Jardiance.

When coverage is available and medically necessary, the dosage, frequency, duration of therapy, and site of care should be reasonable, clinically appropriate, and supported by evidence-based literature and adjusted based upon severity, alternative available treatments, and previous response to therapy.

Receipt of sample product does not satisfy any criteria requirements for coverage.

Conditions Not Covered

Inpefa for any other use is considered not medically necessary, including the following (this list may not be all inclusive; criteria will be updated as new published data are available):

1. **Type 1 Diabetes.** Inpefa is not approved for glycemic control. Note: Patients with heart failure should be reviewed under criteria for *Heart Failure.*

References

1. Inpefa™ tablets [prescribing information]. Lexicon; The Woodlands, TX: January 2024.
2. Heidenreich PA, Bozkurt B, Aguilar D et al. 2022 American Heart Association/American College of Cardiology/Heart Failure Society of America (AHA/ACC/HFSA) guideline for the management of heart failure: a report of the American College of Cardiology/American Heart Association Joint Committee on Clinical Practice Guidelines. *J Card Fail.* 2022;28:e1-e167.
3. American Diabetes Association. Standards of care in diabetes – 2025. Diabetes Care. 2025;48(Suppl 1):S1-S359.
4. Kidney Disease: Improving Global Outcomes (KDIGO) Diabetes Work Group: Rossing P, Muiza Caramori M, Chan JCN, et al. KDIGO 2022 clinical practice guideline for diabetes management in chronic kidney disease. *Kidney Int.* 2022;102(5S):S1-S127.
5. Blonde L, Umpierrez GE, Reddy SS, et al. American Association of Clinical Endocrinology clinical practice guideline: developing a diabetes mellitus comprehensive care plan – 2022 update. *Endocr Pract*. 2022;18:923-1049.
6. Boer IH, Khunti K, Sadusky T, et al. Diabetes management in chronic kidney disease: a consensus report by the American Diabetes Association (ADA) and Kidney Disease: Improving Global Outcomes (KDIGO). *Kidney International.* 2022;102:974-989.
7. Samson SL, Vellanki P, Blonde L, et al. American Association of Clinical Endocrinology consensus statement: comprehensive type 2 diabetes management algorithm – 2023 update. *Endocr Pract.* 2023;29:305-340.

8. Kittleson MM, Panjrath GS, Amancherla K, et al. 2023 ACC expert consensus decision pathway on management of heart failure with preserved ejection fraction. *JACC*. 2023;81(18):1835-1878.
9. Davies MJ, Aroda VR, Collins BS, et al. Management of hyperglycemia in type 2 diabetes, 2022. A consensus report by the American Diabetes Association (ADA) and the European Association for the Study of Diabetes (EASD). *Diabetes Care.* 2022;45:2753-2786.
10. McDonagh TA, Metra M, Adamo M, et al; and the ESC Scientific Document Group. 2023 focused update of the 2021 ESC guidelines for the diagnosis and treatment of acute and chronic heart failure. *Eur Heart J*. 2023;44(37):3627-3639.
11. Maddox TM, Januzzi JL Jr, Allen LA, et al. American College of Cardiology Solution Set Oversight Committee. 2024 ACC expert consensus decision pathway for treatment of heart failure with reduced ejection fraction. *JACC*. 2024;85(15): 1444-1488.

Revision Details

Type of Revision	Summary of Changes	Date
Selected Revision	**Updated** Individual and Family Plan preferred product requirements.	01/01/2025
Annual Revision	**Updated** coverage policy title from "Sotagliflozin" to "Inpefa." **FDA-Approved Indication(s):** **Type 2 Diabetes, To Reduce The Risk of Cardiovascular Death, Hospitalization for Heart Failure, and Urgent Heart Failure Visit.** **Added**, "Note: Patients with heart failure should be reviewed under criteria for *Heart Failure"* for a patient that has one or more cardiovascular risk factors. **Conditions Not Covered:** **Type 1 Diabetes.** **Added**, "Note: Patients with heart failure should be reviewed under criteria for *Heart Failure"*	11/1/2024
Annual Revision	**No criteria changes**	9/15/2025

The policy effective date is in force until updated or retired.

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